

Brunswick Family Medical Centre

COMPREHENSIVE GENERAL PRACTICE · ACCREDITED

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Date: 14 May 2026

Our ref: REF-2026-05-1147

ATTENTION

Prior Authorisation Team

HealthFirst Australia — Diagnostic Imaging Authorisations

GPO Box 7891, Melbourne VIC 3001

Fax: 1300 884 552

Re: Margaret CHEN · DOB 14 June 1968 (57F) · Medicare 2950 41882 1 / 3 · HealthFirst Member No. HF-44782106

PROCEDURE REQUESTED

Procedure	MRI Lumbar Spine — Non-contrast
Procedure code	MRI-LUMBAR (MBS item 63151)
Clinical urgency	Semi-urgent (within 14 days)
Working diagnosis	Right L5/S1 radiculopathy on a background of chronic mechanical low back pain

PRESENTING COMPLAINT & DURATION

Mrs Chen presents with an **8-week history** of low back pain radiating into the right buttock and posterior thigh, worsening over the last fortnight to include calf paraesthesia. Onset was insidious, no specific inciting event. Pain is exacerbated by prolonged sitting and forward flexion.

Duration of symptoms	8 weeks
Current pain score	NPRS 7/10 (worst 9/10, best 4/10)

CONSERVATIVE MANAGEMENT ATTEMPTED

The patient has completed an **8-week course of supervised physiotherapy** at Glenroy Sports & Spine Physiotherapy (Practitioner: Lauren Hayes, APAM Reg. PHY0009482), commencing 17 March 2026 and concluding 12 May 2026. Treatment included:

- Manual therapy — lumbar mobilisations and soft-tissue release (weekly)
- McKenzie-based exercise prescription — repeated extension in lying, progressed to standing
- Dry needling to the right gluteal and paraspinal musculature
- Core stability and neural mobilisation home exercise programme

Outcome: **partial improvement only**. Pain at rest reduced from NPRS 6/10 to 4/10, but radicular pain on loaded flexion and prolonged sitting remains unchanged. Patient also trialled paracetamol 1g QID and a 10-day course of ibuprofen 400mg TDS with limited benefit.

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Physiotherapy duration	8 weeks (17 Mar – 12 May 2026)
Pharmacological trial	Paracetamol, NSAIDs — partial response

EXAMINATION FINDINGS

Neurological examination performed on 12 May 2026:

Motor	Power 5/5 throughout lower limbs bilaterally, including EHL and tibialis anterior
Sensation	Intact to light touch and pin-prick in L4, L5 and S1 dermatomes bilaterally
Reflexes	Knee and ankle reflexes 2+ and symmetric
Provocation	Straight-leg raise positive on the right at 45° reproducing radicular symptoms; negative on the left

RED FLAG SCREENING

No red flags identified. Specifically: no saddle anaesthesia, no bladder or bowel disturbance, no fever, no weight loss, no history of malignancy, no recent trauma, no progressive neurological deficit, no immunosuppression.

REASON FOR IMAGING

Despite an appropriate trial of conservative management, the patient continues to experience significant pain and radicular symptoms with functional impairment (off work for the past three weeks). MRI is requested to characterise suspected lumbar disc pathology and inform the next step in management, including consideration of specialist referral.

Kind regards,

Anita Patel

Dr Anita Patel MBBS, FRACGP
Provider No. 250123BC · Prescriber No. 4451102
Brunswick Family Medical Centre